

D14 — MICU Transfer-In Note

Dr. E. Park, MICU Fellow | MICU | 02/22/2026 18:00

Patient: Pemberton, Harold J. **MRN:** 847-29-3156 **DOB:** 04/12/1951 **Allergies:** Sulfa drugs (rash)

MICU TRANSFER-IN NOTE

Patient: Harold J. Pemberton | MRN: 847-29-3156

Date: 02/22/2026 18:00 | Provider: Dr. E. Park, MICU Fellow

REASON FOR MICU TRANSFER: Hemodynamic instability post-GI hemorrhage and IR embolization

SUMMARY OF EVENTS:

74 y/o male admitted for COPD exacerbation, found to have concurrent acute on chronic CHF exacerbation (EF 30%, BNP 480 → 2,400). Developed upper GI bleed Day 4 (Hgb 11.2 → 7.4) – EGD showed Forrest IIa gastric ulcer, clips placed. Re-bled Day 5 (Hgb 6.2). GI recommended repeat EGD; IR recommended embolization. Attending elected IR embolization – successful selective left gastric artery embolization with Gelfoam. Post-procedure: SBP 84, HR 112.

ACTIVE PROBLEMS:

1. Upper GI hemorrhage – post-IR embolization, monitor for re-bleeding
2. Acute on chronic CHF exacerbation – diuresis interrupted by hemorrhage
3. COPD exacerbation – improving, steroids ongoing
4. AKI on CKD – Cr 1.8 (baseline 1.5), likely pre-renal from hemorrhage
5. T2DM – steroid hyperglycemia
6. Afib – anticoagulation held

CURRENT MEDICATIONS ON TRANSFER:

- Pantoprazole IV drip 8 mg/hr
- Prednisone 40 mg PO daily
- Nebulized albuterol 2.5 mg Q4H
- Nebulized ipratropium 0.5 mg Q6H
- Carvedilol 25 mg PO BID
- Spironolactone 25 mg PO daily
- Lisinopril 20 mg PO daily
- Amlodipine 5 mg PO daily
- Furosemide – HELD (now hypovolemic)
- Apixaban – HELD (GI bleed + post-embolization)
- Metformin – HELD (acute illness)
- Glipizide 5 mg PO BID
- Insulin sliding scale (lispro) QAC/QHS

- Omeprazole – replaced by pantoprazole drip
- Atorvastatin 40 mg PO daily
- Levothyroxine 75 mcg PO daily
- Tamsulosin 0.4 mg PO daily
- Allopurinol 100 mg PO daily
- Docusate 100 mg PO BID
- Senna 8.6 mg PO QHS
- Acetaminophen 500 mg PO Q6H PRN
- Bupropion XL 150 mg PO daily

PLAN:

- Resuscitation: NS boluses PRN, transfuse Hgb < 7 or hemodynamic instability
- Norepinephrine if MAP < 65 despite volume resuscitation
- Serial Hgb Q6H, serial BMP
- Strict I/Os
- Continue pantoprazole drip
- Continue prednisone (cannot abruptly stop)
- Cardiology and GI following

Electronically signed by: Dr. E. Park, MD

Date: 02/22/2026 18:45